Capivasertib + fulvestrant

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Capivasertib + fulvestrant · reviewed support plan

Active medicines: Capivasertib + Fulvestrant

Emetogenic risk: Minimal-to-low oral risk

Medicine	Dose / route	When
Loperamide · treatment of diarrhoea	4 mg orally at first onset, then 2 mg after each loose stool; maximum 16 mg in a day	Use at first onset according to the agreed plan; this is not automatic daily prophylaxis Seek assessment for severe/persistent symptoms, dehydration or inability to drink; confirm individual duration and contraindications.

Additional phase-specific support

- None usually required. Do not import cytotoxic antiemetic dexamethasone/aprepitant routines into this regimen.
- Report new rash promptly. Emollients, antihistamines and topical steroids depend on grade and dermatology advice. Early dermatology advice is recommended. Ongoing antihistamines and/or topical steroids may be considered as secondary prevention after persistent or previous grade 3 rash; this is not mandatory routine prophylaxis for everyone.
- Use the individual written glucose and diabetes-medicine plan. The source suggests metformin on capivasertib dosing days and withdrawal when capivasertib is withdrawn unless otherwise clinically indicated. This must be reconciled with pre-existing diabetes treatment: do not tell all patients to stop their usual diabetes medicines on the three capivasertib-free days. Watch for low glucose as well as high glucose on non-dosing days.
- Strong CYP3A4 inhibitors should be avoided where possible; strong/moderate inhibitors require capivasertib dose review. Strong/moderate CYP3A4 inducers should be avoided. Reconcile medicines affecting narrow-therapeutic-index CYP or UGT substrates and sensitive transport substrates. Dose decisions must preserve earlier toxicity reductions.
- Agree glucose equipment, testing windows, result review and urgent contacts before the first dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00947 v1 · p8 · NCCP oral antiemetic guidance v6, Table 5

Sources checked 2026-10-25 · SACTCheck v0.84.20 · Regimen-level drafts; prescriber and oncology-pharmacy verification required